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Thomas Keane, MD, MBA
Assistant Secretary for Technology Policy
Office of the National Coordinator for Health IT
U.S. Department of Health and Human Services
330 C St SW, Floor 7
Washington, DC 20201

Re: ***Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care***

Dear Assistant Secretary Keane,

On behalf of OCHIN, thank you for the opportunity to comment on the Department of Health and Human Services' (HHS) Request for Information on "Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care." Artificial intelligence (AI) has the potential to support earlier intervention, improve chronic disease management, and increase efficiency across care delivery. Achieving these outcomes at scale, however, requires that AI adoption be **affordable, trusted, and grounded in evidence generated through routine clinical care, particularly among rural and low-resourced providers**.

For rural and low-resourced providers, affordability is inseparable from sustainable infrastructure. AI adoption depends not only on the maturity of algorithms, but on access to modern health IT systems, shared-service operating models, strong governance, and a workforce prepared for new digital roles—capabilities that are often cost-prohibitive to build independently. Trust in clinical AI must be established through real-world evidence generated in local care settings, allowing performance, safety, and workflow impact to be validated over time rather than inferred from isolated pilots. Without this foundation, AI risks increasing fragmentation rather than productivity, particularly among providers in rural and low resourced settings.

OCHIN: Shared Infrastructure Built for Trust, Affordability, and Scale

OCHIN is a nonprofit shared health IT and services organization supporting more than 300 independent providers across 44 states, including rural health clinics, federally qualified health centers, tribal clinics, and critical access hospitals. We support nearly 44,000 providers nationwide, generating real-world evidence from routine clinical care across rural, tribal, and other historically underserved communities. Our experience demonstrates that AI tools that perform well in rural and low-resourced settings—where staffing is limited, workflows are stretched, and tolerance for disruption is low—represent the true value proposition of clinical AI for the healthcare system as a whole. In October 2025 alone, OCHIN network members used generative AI features nearly 700,000 times, and by the end of 2025, more than 20 AI-enabled tools were live across the network.

Our shared-services model enables thin-margin providers in rural and least-resourced communities to access enterprise-grade technology, cybersecurity, compliance, governance, and workforce training at a fraction of the cost of building and maintaining those capabilities independently. Fixed costs are spread across a national network. Duplication is reduced. Accountability is strengthened through centralized

governance where appropriate. Federal investments go further. This shared infrastructure is fundamental to affordability at both the provider and system level. For rural and low-resourced organizations, shared services are not optional enhancements; they are the only viable path to sustaining modern health IT and responsible AI adoption without diverting scarce resources away from patient care. Our learnings from AI adoption in the OCHIN network are included in the Appendix.

Recommendations

We respectfully recommend that **HHS prioritize the following policies for rural and low-resourced settings**, leveraging existing trusted partners with established track records and mature operating models:

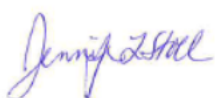
- **Coordinate across HHS agencies and with the Department of Labor to fund health IT workforce development and AI skilling.** As care delivery becomes increasingly digital, AI adoption depends on a dependable operating backbone and a workforce equipped to implement, govern, and monitor AI across clinical and operational workflows. Targeted investment enables AI to function as a true capacity multiplier.
- **Underwrite shared-service models that support the validation, deployment, and ongoing monitoring of clinical AI.** Federal investment in network-based AI services can reduce duplicative cost and governance burden, enable consistent and locally appropriate evaluation of AI in real-world care settings, and create economies of scale that accelerate responsible adoption.
- **Invest in modernized health IT systems for rural and low-resourced providers operating on antiquated platforms.** AI-enabled tools depend on interoperable data exchange, secure infrastructure, and modern core systems. Targeted modernization investment reduces cybersecurity risk and ensures AI can be deployed safely where need is greatest.
- **Advance regional technical assistance resource centers to support AI adoption in clinical care.** Even when tools and infrastructure exist, many providers lack hands-on expertise to integrate AI into workflows and navigate evolving regulatory requirements. Regional centers can accelerate learning and reduce implementation risk.

Conclusion

Healthcare is entering a period defined by constrained resources and increasing demand. The question is not whether AI will be adopted, but whether it will be deployed in ways that increase productivity, strengthen rural access, and improve long-term outcomes. That outcome depends on AI adoption grounded in affordable, trusted infrastructure and evidence generated through real-world clinical use, particularly in rural and low-resourced settings that test whether innovation truly delivers value. We appreciate the opportunity to provide input and welcome continued partnership in advancing infrastructure-based AI adoption that delivers measurable value for patients, providers, and taxpayers alike.

Please contact me at stollj@ochin.org if we can provide any additional information support your efforts.

Sincerely,



Jennifer Stoll
Chief External Affairs Officer

APPENDIX

Operating at national scale also enables OCHIN to serve as a trusted partner by generating evidence through routine clinical use. Because AI-enabled capabilities are deployed across a diverse network of providers, performance, workflow impact, and operational risk can be observed and validated over time in real-world care settings. This shared infrastructure supports consistent implementation and centralized oversight, while enabling local validation within the clinical environments where AI is actually used.

Trust, Local Validation, and the Rural Value Proposition

Trust in clinical AI depends on evidence generated in the populations and workflows where tools are deployed. Shared-service models support local validation within a consistent governance framework, allowing performance, safety, and workflow impact to be evaluated over time without duplicative, site-by-site build. Validation, oversight, data stewardship, and regulatory alignment are supported centrally, while preserving local clinical autonomy.

Within this infrastructure, AI tools are embedded directly into clinical and operational workflows supporting chronic disease risk stratification, care coordination, documentation efficiency, revenue cycle integrity, and population health management. These use cases are tested daily in rural and low-resourced environments where staffing shortages, broadband constraints, and complex patient needs magnify both the risks and rewards of AI adoption. When AI tools demonstrate value in these settings by reducing burden, improving access, and supporting clinical decision-making, they are more likely to scale safely and sustainably across the broader healthcare system.

What We Have Learned

- **Trust in clinical AI is built through evidence generated under local conditions.** Providers, patients, and regulators gain confidence when AI performance is validated in care settings serving similar populations, rather than extrapolated from pilots in well-resourced systems.
- **Healthcare faces sustained fiscal and workforce constraints alongside growing demand.** AI must help the system do more with less by increasing productivity, not by preserving legacy administrative structures. AI adoption changes work. It does not eliminate the need for people, but it does require new competencies in AI oversight, workflow redesign, data governance, cybersecurity, preventive care coordination, and digital population health management.
- **Organizational capacity, not algorithm maturity alone, determines whether AI reduces burden or adds complexity.** Rural and low-resourced providers face the greatest constraints in margins and staffing, limiting their ability to independently build the operational, governance, and validation capabilities required for sustained AI use.
- **Shared workforce development models scale effectively when delivered through trusted infrastructure.** Through the federally supported Health Center Controlled Network (HCCN) program and national digital health training initiatives, OCHIN delivers health IT and AI workforce development across rural and low-resourced care settings, demonstrating that workforce readiness can be advanced efficiently through shared models rather than site-by-site investment.

Lowering Costs by Leveraging AI Tools to Optimize Clinical Documentation

OCHIN evaluated two AI-enabled ambient documentation tools and found positive ROI, with observed improvements in documentation efficiency, clinician productivity, and workflow performance.

- **DAX Copilot:** Estimated **return of \$5.50 per every \$1 invested**, assuming a 70% reduction in burnout and 50% reduction in documentation time.
- **Abridge:** Estimated return of **\$5 per every \$1 invested**, assuming a 60% reduction in documentation time and a 10% E&M undercoding rate.

- **Strategic federal investment in workforce transition and AI skilling is productivity policy, not job preservation policy.** Targeted investment ensures that AI adoption strengthens access and long-term sustainability rather than increasing operational strain or widening gaps for providers with the least capacity to absorb change.